

Study C906289002

Mock Phase 2 Oncology Trial

Protocol Version 1.0 · Synthetic document for demonstration only.

1. Background

Study C906289002 is a Mock Phase 2 oncology trial evaluating investigational agent MX-209 in adults with relapsed or refractory solid tumors. Prior studies demonstrated single-agent activity and an acceptable safety profile at the recommended Phase 2 dose. This protocol is provided for demonstration purposes only and does not describe a real clinical investigation.

The investigational product targets a tumor-associated signaling pathway that has been implicated in proliferation and resistance to standard cytotoxic therapy. Preclinical models indicated tumor growth inhibition across multiple histologies.

2. Objectives

The primary objective of Study C906289002 is to estimate the objective response rate per RECIST v1.1 in the eligible population. Key secondary objectives include progression-free survival, overall survival, duration of response, and characterization of the safety and tolerability profile.

Exploratory objectives include pharmacokinetic characterization and assessment of candidate biomarkers of response and resistance.

3. Study Design

This is an open-label, single-arm, multicenter Phase 2 study. Approximately 80 subjects will be enrolled across 12 sites in North America and Europe. Subjects will receive MX-209 in 28-day cycles until disease progression, unacceptable toxicity, or withdrawal of consent.

Tumor assessments will be performed at screening and every two cycles (8 weeks). End-of-treatment and 30-day safety follow-up visits are required for all subjects.

4. Eligibility

Subjects must be at least 18 years of age with histologically or cytologically confirmed solid tumor that has progressed on or after standard therapy. ECOG performance status of 0 or 1 is required. Adequate hematologic, hepatic, and renal function as defined by the laboratory criteria in Appendix A must be documented within 14 days prior to enrollment.

Key exclusion criteria include active central nervous system metastases, uncontrolled intercurrent illness, pregnancy or lactation, and receipt of any investigational therapy within 28 days prior to the first dose of study drug.

5. Dosing

MX-209 will be administered orally at the recommended Phase 2 dose of 200 mg once daily on a continuous schedule. Doses should be taken at approximately the same time each day with or without food. Missed doses may be taken within 6 hours of the scheduled time; otherwise the dose should be skipped.

Dose modifications for treatment-emergent toxicity follow the schema in Appendix B. Two dose reductions are permitted (to 150 mg and 100 mg). Subjects requiring a third dose reduction will be discontinued from study treatment.

6. Adverse Event Definitions

An adverse event (AE) is any untoward medical occurrence in a subject administered the investigational product, whether or not considered related to the product. AEs will be graded according to NCI CTCAE v5.0 and coded using the latest version of MedDRA.

A serious adverse event (SAE) is any AE that results in death, is life threatening, requires inpatient hospitalization or prolongation of existing hospitalization, results in persistent or significant disability or incapacity, is a congenital anomaly or birth defect, or is an important medical event based on medical judgment.

Adverse events of special interest (AESIs) for this protocol include febrile neutropenia, Grade 3 or higher hepatic enzyme elevations, and any infection of Grade 3 or higher severity.

7. Safety Monitoring

All subjects will be monitored continuously for AEs from the time of first dose through 30 days after the last dose. Investigators will assess and grade events at each scheduled visit and as clinically indicated.

Serious adverse events must be reported to the Sponsor within 24 hours of investigator awareness, regardless of causality. The Sponsor will forward qualifying events to applicable regulatory authorities in accordance with local expedited reporting requirements.

An independent Data Monitoring Committee will review aggregate safety data at pre-specified intervals and may recommend protocol modifications or study suspension based on emerging safety signals.

8. Statistical Analysis

The primary analysis population for efficacy is the Full Analysis Set, defined as all enrolled subjects who receive at least one dose of MX-209. The Safety Analysis Set is identical to the Full Analysis Set for this protocol.

Objective response rate will be summarized with a two-sided 95% Clopper-Pearson confidence interval. Time-to-event endpoints will be summarized using Kaplan-Meier methodology. No formal hypothesis testing is planned.